

THE RESULTS OF YOUR ANALYSIS

PREPARED FOR

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DATE OF IMAGES : 7/26/2016

DATE OF ANALYSIS : 7/26/2016

REFERRING DOCTOR :

Dr. David Bohn

This report contains important information concerning structural changes that can be affecting your overall level of health and well-being. Spinal stability is a basic requirement for the protection of your nervous structures and the prevention of early mechanical deterioration of your spinal component

Instability is generally considered to be a global increase in the movements associated with the occurrence of back, neck, and/or nerve root pain. Damage to any spinal structure produces some degree of spinal instability.

This computer aided digital analysis is an overview of your current level of spinal stability.

APOM Left / Right Lateral

PATIENT RESULT



PATIENT RESULT



The RED line denotes the lateral mass of the Atlas (C1) in lateral flexion.

The GREEN line denotes the most lateral point of the superior articular process of Axis (C2).

A distance of more than 1.7 mm of the red line from the green line indicates subluxation. When the lateral shift of the red line from the green line is greater than 1.7 mm, clinical correlation is recommended.

A lateral distance of more than 3.0 mm of the red line from the green line indicates probable Alar, Transverse, and/or Accessory ligament damage.

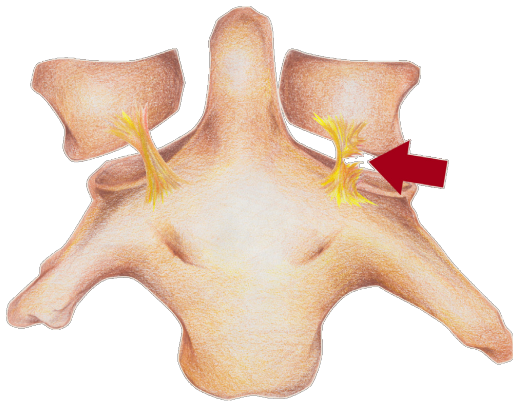
C1-C2 Left Lateral Translation

2.24 mm - Subluxation

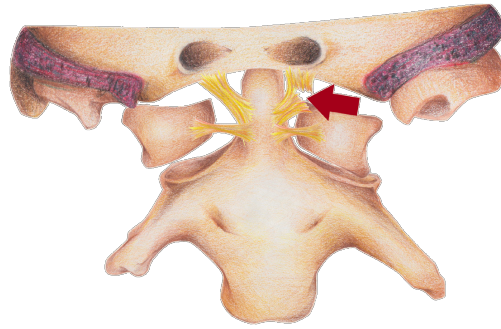
C1-C2 right Lateral Translation

4.18 mm - Instability Likely

Accessory Ligament Failure



Alar/Transverse Ligament Failure



ADDENDUM:

1. Measurements greater than 1mm Translation and/or more than 7 degrees of Angular Variation are deemed to be clinically significant and in excess of normal flexibility of the cervical spine.

~ Lin, Tsai, Chu and Chang. SPINE 2001, February; 26(3): 256-261,

2. Abnormal measurements greater than 11 degrees of Angular Variation and/or more than or equal to 3.5mm of lateral Translation (Loss of Motion Segment Integrity) are by definition an indication of ligament damage which results in segmental instability and implies a whole person impairment.

~Guides to the Evaluation of Permanent Impairment, 5th Edition, AMA, 2000

3. Lateral shift of Atlas on Axis greater than 1.7mm is considered subluxation and is associated with a poor prognosis after whiplash injury.

~Krakenes J, Kaale BR, Moen G, Nordli H, Gilhus NE, Rovik J. MRI assessment of the alar ligaments in the late state of whiplash injury-structural abnormalities and observer agreement. Neuroradiology 2002 Jul;44(7): 617-24.

IMPAIRMENT RATING:

NOTE: SpineTech Pro software (CRMA) correlates with the AMA Guidelines 4th Edition (p.98-99, 1 09), 51h Edition (p.378-79 and the 6th Edition. 578-79, 564). The 6th Edition requires that objective evidence be utilized when evaluating permanent impairment. SpineTech Pro software uses X-ray image overlays technology and qualifies as "Evidence Based Objective Impairment Rating". This software is designed to detect and evaluate ratable impairment established by the AMA Guidelines.